

Diet for Gout Patients: Recommended Foods, Dietary Patterns, and Foods to Avoid

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Background: diet goals in gout management

The main diet goal in gout is to lower uric acid and reduce gout flares. Diet helps, but it works best along with weight control, good hydration, and medicines when needed. (LLM Memory)

Gout happens when uric acid builds up and forms crystals in joints, so diet aims to reduce the body's uric acid load and lower the chance of crystal formation and flares. In practice, this means choosing an eating pattern that limits high-purine foods, avoids excess alcohol and sugar-sweetened drinks, supports a healthy body weight, and is easy to keep up over time rather than using short-term restrictive diets. Weight loss can help lower uric acid in people who are overweight, but crash dieting or fasting can make gout worse because rapid breakdown of body tissue can raise uric acid. Diet alone usually lowers uric acid only modestly, so many patients still need urate-lowering medicine; the goal is to use diet as one part of long-term gout control, not as the only treatment. (Model-Generated)

Recommended dietary patterns and foods to encourage

For most gout patients, the best diet is not an extreme "gout diet," but a healthy pattern like DASH or Mediterranean-style eating. In practice, that means mostly plant foods, low-fat dairy, healthier protein choices, enough water, and a few foods such as cherries that may help lower flare risk. (28 sources)

Rather than focusing only on single foods, many papers support using an overall healthy eating pattern such as the DASH diet or a Mediterranean-style diet, both of which emphasize fruits, vegetables, whole grains, and low-fat dairy while lowering saturated fat, added sugar, and red/processed meat intake. These patterns have been linked with lower serum urate and lower gout risk, and are often suggested for patients with mild gout or hyperuricemia, especially when gout overlaps with obesity, hypertension, or other metabolic problems. ([M. et al., 2020](#)) ([Zhang et al., 2022](#)) ([Tomaszuk et al., 2023](#)) ([Li, 2024](#)) ([Sun et al., 2025](#)) ([Tang et al., 2017](#)) ([Chen et al., 2025](#)) ([Yokose et al., 2022](#)) ([Rai et al., 2017](#)) ([Kwiatkowska et al., 2020](#)) ([Ayoub-Charette et al., 2019](#)) ([Stamostergiou et al., 2018](#)) ([Zhang et al., 2019](#))

Foods to actively include are low-fat or nonfat dairy products such as skim milk and low-fat yogurt, which are repeatedly associated with lower uric acid or lower gout risk. Several reviews and guidelines also support getting protein more often from plant sources, including beans, lentils, legumes, and soy, instead of relying mainly on meat and seafood. ([Singh et al., 2016](#)) ([Bitik et al., 2014](#)) ([Zgaga et al., 2012](#)) ([Raziel, 2018](#)) ([Hui et al., 2017](#)) ([Filip et al., 2020](#)) ([Hui et al., 2017](#)) ([Yip et al., 2023](#)) ([Hassan et al., 2023](#)) ([Mei et al., 2017](#)) ([Li et al., 2025](#))

Vegetables, fruit, whole grains, nuts, and fiber-rich foods fit well in a gout-friendly diet. Importantly, plant

foods that contain purines do not seem to raise gout risk the way animal purines do, so most patients do not need to avoid purine-rich vegetables; encouraging vegetable intake is supported by several sources. ([Sahai et al., 2019](#)) ([Bitik et al., 2014](#)) ([Zgaga et al., 2012](#)) ([Mei et al., 2017](#)) ([Liang et al., 2024](#)) ([Li et al., 2025](#))

Some specific foods may offer added benefit. Cherry intake has been associated with fewer recurrent gout attacks, and some guidelines and reviews recommend including cherries as part of the diet. Vitamin C intake has also been linked with lower uric acid or lower gout risk, though it is usually best approached through food first unless a clinician advises supplements. ([Bitik et al., 2014](#)) ([Zhang et al., 2012](#)) ([Raziel, 2018](#)) ([Hui et al., 2017](#)) ([Filip et al., 2020](#)) ([Hui et al., 2017](#)) ([Singh et al., 2016](#)) ([Choi et al., 2009](#)) ([Mei et al., 2017](#))

For drinks, plain water is the safest default, and good fluid intake is commonly advised. Some reports also note possible benefit from coffee, but this is a secondary point compared with the better-supported advice to use water, avoid sugary drinks, and build meals around healthy overall patterns. ([Sahai et al., 2019](#)) ([Chonin et al., 2021](#)) ([Bitik et al., 2014](#))

Evidence

(M. et al., 2020)

[Comparative Study of Uric Acid Levels between Rural and Urban Populations](#)

"Hyperuricaemic patients are recommended to reduce food causing hyperuricemia such as high purine rich diets, sweetened and alcoholic beverages"

".Following DASH diet (dietary approaches to stop hypertension) help in decreasing uric acid levels in serum. This diet helps in intake of fruits, vegetables, low fat products, whole grains, limiting sugars, sweetened beverages and saturated fats."

(Zhang et al., 2022)

[Gout and Diet: A Comprehensive Review of Mechanisms and Management](#)

"The typical dietary patterns include a DASH and Mediterranean diet, both of which are comprised of fruits, vegetables, and low-fat dairy products with reductions in total and saturated fats"

". Therefore, our recommendation is for individuals to follow a healthy diet for prevention purposes, and for patients with mild gout, we recommend the DASH and Mediterranean diet, which focus on plant-based components. Additionally, we recommend a reduction in the consumption of high-fat foods (fast food and cream products), especially foods with trans fatty acids (such as margarine and butter), and for individuals to pay attention to the amount of nutrient supplements consumed."

(Tomaszuk et al., 2023)

[Nutritional factors in the prevention and treatment of gout](#)

"Studies to date suggest that avoidance of certain foods and beverages can decrease the frequency of gout flares. Weight loss may be beneficial for prevention as well as treatment of gout and its comorbidities. The impact of various types of diet on the course of gout has been given particular attention and recent research suggests that vegetarian, mediterranean and dash diets may be beneficial for gouty patients."

(Li, 2024)

[Study on the Role of Diet in Gout Management and Its Mechanism](#)

"Current research suggests that a low-purine diet, as well as avoiding alcohol intake, can help lower uric acid levels and reduce gout attacks"

". The results of the study showed that the Mediterranean diet, the DASH diet, was more helpful in reducing uric acid levels."

(Sun et al., 2025)

[2024 Update of Chinese Guidelines for Diagnosis and Treatment of Hyperuricemia and Gout Part I: Recommendations for General Patients](#)

"Dietary restrictions on high-purine foods, high-fructose products, and alcohol are recommended; while dietary fiber, tart cherry products, citric acid products, and skim milk are encouraged"

".Certain dietary patterns, such as the Mediterranean diet and DASH diet (the dietary pattern in Dietary Approaches to Stop Hypertension), are beneficial for patients with gout by improving hyperuricemia and metabolic syndrome. These diets are more

beneficial compared to the western diet, which is characterized by high purine, fat, and calorie content (Tang et al., 2017)(Lubawy et al., 2023)."

(Tang et al., 2017)

DASH Diet and Change in Serum Uric Acid over Time

"The Dietary Approaches to Stop Hypertension (DASH) diet lowers serum uric acid (SUA) levels compared to the typical American (control) diet. However, the time required for the DASH diet to take effect is unknown. We analyzed data from a parallel arm, randomized-controlled trial in pre-hypertensive or hypertensive adults (N = 103), comparing the effects of DASH or a control diet on SUA measured at 30, 60, and 90 days. Effects were examined overall and within stratified subgroups based on baseline SUA status (SUA ≥ 6 mg/dL vs < 6 mg/dL). The mean age of participants was 51.5 ± 9.7 years, 55% were women, 75% were black, 42% were obese, and 34% had hypertension. Twenty-four of the 103 (23%) participants had a baseline SUA ≥ 6 mg/dL. Overall, compared to the control, DASH lowered SUA by 0.5 mg/dL at 30 and 90 days. Among participants with baseline SUA ≥ 6 mg/dL, DASH lowered SUA by 0.8 and 1.0 mg/dL at 30 and 90 days, respectively. These findings demonstrate that the DASH diet reduces SUA within 30 days, with a sustained effect at 90 days, which is informative for healthcare providers counseling patients on time course expectations for uric acid reduction in response to dietary modification."

(Chen et al., 2025)

Advances in Understanding the Mechanisms of Treatment for Gouty Arthritis: A Comprehensive Review

"Traditional dietary recommendations for gout have focused on limiting intake of purine-rich foods -such as organ meats, certain seafood, and beer -to reduce serum urate levels (Nielsen et al., 2018). While it remains important for individuals with gout to recognize and avoid foods that may specifically trigger their flares, emerging evidence indicates that the overall dietary pattern may be more beneficial than strict purine restriction alone"

".Recent large-scale studies and expert guidelines now favor the adoption of cardiometabolically healthy dietary patterns such as the Dietary Approaches to Stop Hypertension (DASH) diet and the Alternative Healthy Eating Index (AHEI), which are associated with lower serum urate levels, reduced risk of gout, and improvement in comorbidities like hypertension and insulin resistance (Rai et al., 2017)(Yokose et al., 2022). The Mediterranean diet, which emphasizes fruit, vegetables, whole grains, healthy fats, and moderate dairy intake, has also been suggested as beneficial in observational studies (Dessein et al., 2000)"

".Alcohol consumption: Avoidance of alcohol, particularly beer and spirits, is a key aspect of dietary management in gout."

(Yokose et al., 2022)

Adherence to 2020 to 2025 Dietary Guidelines for Americans and the Risk of New-Onset Female Gout.

Importance

Female-specific gout data are scarce despite perceived differences from males in its risk factors and disproportionate worsening in disease and comorbidity burden globally. The 2020 to 2025 Dietary Guidelines for Americans recommend multiple healthy eating patterns for prevention of cardiovascular-metabolic outcomes, which may also be relevant to the prevention of female gout.

Objective

To examine the associations of dietary scores for the latest guideline-based healthy eating patterns with risk of incident female gout.

Design, Setting, and Participants

This prospective cohort study included 80 039 US women in the Nurses' Health Study followed up through questionnaires every 2 years starting from 1984. Participants had no history of gout at baseline, and the study used questionnaire responses through 2018. Statistical analyses were performed over September 2020 to August 2021.

Exposures

Four healthy eating patterns: Dietary Approaches to Stop Hypertension (DASH), Alternate Mediterranean Diet Score, Alternative Healthy Eating Index (AHEI), and Prudent, plus Western (unhealthy) for comparison, with scores derived from validated food frequency questionnaires.

Main Outcomes and Measures

Incident, physician-diagnosed female-specific gout.

Results

During 34 years of follow-up, we documented 3890 cases of incident female gout. Compared with the least-adherent quintile, women most adherent to healthy diets had significantly lower risk of incident gout, with multivariable-adjusted hazard ratios (HRs) 0.68 (95% CI, 0.61-0.76) (DASH), 0.88 (95% CI, 0.80-0.98) (Mediterranean), 0.79 (95% CI, 0.71-0.89) (AHEI), and 0.75 (95% CI, 0.73-0.90) (Prudent); all P for trend $\leq .009$. Conversely, women with highest-quintile Western diet score had 49% higher risk of gout (HR, 1.49; 95% CI, 1.33-1.68], $P < .001$). When combined, the most DASH-diet adherent women with normal body mass index (BMI; calculated as weight in kilograms divided by height in meters squared) had a 68% lower risk of gout compared with the least adherent women with overweight or obese BMI; the corresponding risk reduction was 65% combining high DASH diet adherence with no diuretic use.

Conclusions and Relevance

These large-scale, long-term prospective cohort findings extend the pleiotropic benefits of the 2020 to 2025 Dietary Guidelines for Americans to female gout prevention, with multiple healthy diets that can be adapted to individual food traditions, preferences, and comorbidities."

(Rai et al., 2017)

The Dietary Approaches to Stop Hypertension (DASH) diet, Western diet, and risk of gout in men: prospective cohort study

"Objective To prospectively examine the relation between the Dietary Approaches to Stop Hypertension (DASH) and Western diets and risk of gout (ie, the clinical endpoint of hyperuricemia) in men. Design Prospective cohort study. Setting The Health Professionals Follow-up Study. Participants 44 444 men with no history of gout at baseline. Using validated food frequency questionnaires, each participant was assigned a DASH dietary pattern score (based on high intake of fruits, vegetables, nuts and legumes, low fat dairy products, and whole grains, and low intake of sodium, sweetened beverages, and red and processed meats) and a Western dietary pattern score (based on high intake of red and processed meats, French fries, refined grains, sweets, and desserts). Main outcome measure Risk of incident gout meeting the preliminary American College of Rheumatology survey criteria for gout, adjusting for potential confounders, including age, body mass index, hypertension, diuretic use, and alcohol intake. Results During 26 years of follow-up, 1731 confirmed cases of incident gout were documented. A higher DASH dietary pattern score was associated with a lower risk for gout (adjusted relative risk for extreme fifths 0.68, 95% confidence interval 0.57 to 0.80, P value for trend <0.001). In contrast, a higher Western dietary pattern score was associated with an increased risk for gout (1.42, 1.16 to 1.74, $P=0.005$). Conclusion The DASH diet is associated with a lower risk of gout, suggesting that its effect of lowering uric acid levels in individuals with hyperuricemia translates to a lower risk of gout. Conversely, the Western diet is associated with a higher risk of gout. The DASH diet may provide an attractive preventive dietary approach for men at risk of gout."

(Kwiatkowska et al., 2020)

The place of omega-3 and omega-6 acids in supplementary treatment of inflammatory joint diseases

"Treatment for gout usually includes pharmacological treatment and implementation of a diet aiming to reduce the purine content in consumed meals. Recent studies have shown that, similarly to other rheumatic diseases, the Mediterranean diet, namely a diet rich in polyunsaturated fatty acids, influences the level of uric acid and decreases the frequency of gout attacks (Stamostergiou et al., 2018)"

".The recently observed increasing number of patients suffering from gout results from the growing number of obese people consuming excessive amounts of meat, as well as from rising consumption of fructose (Ayoub-Charette et al., 2019)"

".Nonetheless, it is worth remembering the recommendation to limit the overall purine intake (Zhang et al., 2019)."

(Ayoub-Charette et al., 2019)

Important food sources of fructose-containing sugars and incident gout: a systematic review and meta-analysis of prospective cohort studies

"Objective Sugar-sweetened beverages (SSBs) are associated with hyperuricaemia and gout. Whether other important food sources of fructose-containing sugars share this association is unclear. Design To assess the relation of important food sources of fructose-containing sugars with incident gout and hyperuricaemia, we conducted a systematic review and meta-analysis of prospective cohort studies. Methods We searched MEDLINE, Embase and the Cochrane Library (through 13 September 2017). We included prospective cohort studies that investigated the relationship between food sources of sugar and incident gout or hyperuricaemia. Two independent reviewers extracted relevant data and assessed the risk of bias. We pooled natural-log transformed risk ratios (RRs) using the generic inverse variance method with random effects model and expressed as RR with 95% confidence intervals (CIs). The overall certainty of the evidence was assessed using the Grading of Recommendations Assessment, Development and Evaluation system. Results We identified three studies (1 54 289 participants, 1761 cases of gout), comparing the highest with the lowest level of exposure for SSBs, fruit juices and fruits. No reports were found reporting incident hyperuricaemia. Fruit juice and SSB intake showed an adverse association (fruit juice: RR=1.77, 95% CI 1.20 to 2.61; SSB: RR=2.08, 95% CI 1.40 to 3.08), when comparing the highest to lowest intake of the most adjusted models. There was no significant association between fruit intake and gout (RR 0.85, 95% CI 0.63 to 1.14). The strongest evidence was for the adverse association with SSB intake (moderate certainty), and the weakest evidence was for the adverse association with fruit juice intake (very low certainty) and lack of association with

fruit intake (very low certainty). Conclusion There is an adverse association of SSB and fruit juice intake with incident gout, which does not appear to extend to fruit intake. Further research is needed to improve our estimates. Trial registration number NCT02702375; Results."

(Stamostergiou et al., 2018)

The role of the Mediterranean diet in hyperuricemia and gout

"The effect of diet habits in the induction of hyperuricemia and gout is extensively studied and several nutritional factors exacerbating the disease have been identified. In this review, we discuss the data so far obtained of the beneficial role on controlling hyperuricemia of Mediterranean diet, which is full of mono-unsaturated fatty acids and flavonoids and sort of butter, processed food and red meat. We emphasize that though the published findings are promising the data are limited and more studies are needed."

(Zhang et al., 2019)

Effect of Dietary and Supplemental Omega-3 Polyunsaturated Fatty Acids on Risk of Recurrent Gout Flares

"To determine the relationship between omega-3 polyunsaturated fatty acid (n-3 PUFA) consumption (dietary or supplemental) and risk of gout flares."

(Singh et al., 2016)

A cross-sectional internet-based patient survey of the management strategies for gout

"Dietary modifications recommended for gout include increase in low-fat dairy and Vitamin C, and reduction of total protein, alcohol intake and high-fructose drinks (Choi, 2010)(Choi, 2005)."

(Bitik et al., 2014)

An old disease with new insights: Update on diagnosis and treatment of gout.

"The association between a chronic purine-rich diet, mainly of animal origin, and hyperuricemia or incident gout is well established (Choi et al., 2005)(Villegas et al., 2012)(Choi et al., 2004)(Kaneko et al., 2014). It is also known that acute purine intake (over 2 days) increases the risk of recurrent gout attacks in patients with gout (Zhang et al., 2012). Fructose-rich beverages, such as sugar-sweetened soda, are reported to increase the risk of incident gout (Choi et al., 2010). Alcohol (including wine, beer, and liquor), sugar-sweetened soft drinks and, fructose consumption were found to be associated with an increased risk of gout (Choi et al., 2004)(Neogi et al., 2014)(Choi et al., 2008). Some foods and beverages are reported to have protective effects against gout. Dairy products, cherry consumption, or vitamin C intake are reported to decrease serum uric acid and the frequency of gout flares (Choi et al., 2005)(Zgaga et al., 2012)(Dalbeth et al., 2011)(Dalbeth et al., 2012)(Choi et al., 2009)(Zhang et al., 2012). The intake of purine-rich vegetables was not associated to plasma uric acid (Zgaga et al., 2012). A large prospective observational trial found that the risk of incident gout decreased with increasing coffee intake (>4-5 cups per day) (Choi et al., 2007)."

(Zgaga et al., 2012)

The Association of Dietary Intake of Purine-Rich Vegetables, Sugar-Sweetened Beverages and Dairy with Plasma Urate, in a Cross-Sectional Study

"Introduction Hyperuricemia is a strong risk factor for gout. The incidence of gout and hyperuricemia has increased recently, which is thought to be, in part, due to changes in diet and lifestyle. Objective of this study was to investigate the association between plasma urate concentration and: a) food items: dairy, sugar-sweetened beverages (SSB) and purine-rich vegetables; b) related nutrients: lactose, calcium and fructose. Methods A total of 2,076 healthy participants (44% female) from a population-based case-control study in Scotland (1999–2006) were included in this study. Dietary data was collected using a semi-quantitative food frequency questionnaire (FFQ). Nutrient intake was calculated using FFQ and composition of foods information. Urate concentration was measured in plasma. Results Mean urate concentration was 283.8±72.1 mmol/dL (females: 260.1±68.9 mmol/dL and males: 302.3±69.2 mmol/dL). Using multivariate regression analysis we found that dairy, calcium and lactose intakes were inversely associated with urate ($p=0.008$, $p=0.003$, $p=0.0007$, respectively). Overall SSB consumption was positively associated with urate ($p=0.008$), however, energy-adjusted fructose intake was not associated with urate ($p=0.66$). The intake of purine-rich vegetables was not associated to plasma urate ($p=0.38$). Conclusions Our results suggest that limiting purine-rich vegetables intake for lowering plasma urate may be ineffectual, despite current recommendations. Although a positive association between plasma urate and SSB consumption was found, there was no association with fructose intake, suggesting that fructose is not the causal agent underlying the SSB-urate association. The abundant evidence supporting the inverse association between plasma urate concentration and dairy consumption should be reflected in dietary guidelines for hyperuricemic individuals and gout patients. Further research is needed to establish which nutrients and food products influence plasma urate concentration, to inform the development of evidence-based dietary guidelines."

(Raziel, 2018)

Gout, obesity and bariatric surgery

"According to the British Society for Rheumatology Guideline for the Management of Gout from 2017, a well-balanced low in fat and

sugars diet, and high in vegetables and fiber should be encouraged; excessive intake of alcoholic drinks and high purine foods should be avoided; inclusion of soy beans and vegetable sources of protein, and cherries in the diet are recommended. (Hui et al., 2017)"

(Hui et al., 2017)

The British Society for Rheumatology Guideline for the Management of Gout.

"Gout is the most common cause of inflammatory arthritis worldwide. In UK general practice, the overall prevalence has increased from 1.4% in 1999 to 2.49% in 2012 [1], despite the availability of effective and potentially curative urate-lowering drugs for >50 years and evidence-based British and European management guidelines for nearly a decade [2, 3]. Clinical manifestations of gout resulting from monosodium urate crystal deposition include tophi, chronic arthritis, urolithiasis and renal disease as well as recurrent acute arthritis, bursitis and cellulitis. Gouty arthritis and tophi are associated with chronic disability, impairment of health-related quality of life [4-7], increased use of healthcare resources and reduced productivity [8]. Gout is also frequently associated with co-morbidities such as obesity, dyslipidaemia, diabetes mellitus, chronic renal insufficiency, hypertension, cardiovascular disease, hypothyroidism, anaemia, psoriasis, chronic pulmonary diseases, depression and OA [1] as well as with an increase in all-cause mortality (adjusted hazard ratio 1.13, 95% CI: 1.08, 1.18) and urogenital malignancy [1, 9]. Sustained hyperuricaemia is the single most important risk factor for the development of gout. Hyperuricaemia occurs secondarily to reduced fractional clearance of uric acid in > 90% of patients with gout [10]. Age, male gender, menopausal status in females, impairment of"

(Filip et al., 2020)

Gout – management of a chronic disease: a systematic review

"Patients with gout should be encouraged to have a well-balanced diet, rich in vegetables and fiber, low in fat and sugar and also to exercise.

The major risk factor for gout in men is weight gain so that weight loss is necessary, the same as stopping the use of high-fructose corn syrup, rich purine animal protein (organ meats, lamb, pork, beef, shellfish) and alcohol (Hainer et al., 2014). The diet is based on skim milk and/ or lowfat yogurt, vegetable sources of protein, including beans and soy, but also cherries (Hui et al., 2017)."

(Yip et al., 2023)

The Hong Kong Society of Rheumatology consensus recommendations for the management of gout

"Patients should be advised to avoid alcohol, fructose, or sugar-sweetened beverages, and decrease dietary protein source from meat and seafood"

". A diet that restricts alcohol and purine-rich and fructose-rich foods, while maintaining the intake of low-fat dairy products, healthy protein sources, and vegetables, has been observed to lower the risk of incident gout [122]."

(Hassan et al., 2023)

Gout: A Reviewing of The Recent Literature

"To reduce the likelihood of recurrent flares, patients should limit their consumption of certain purine-rich foods (e.g., organ meats, shellfish) and avoid alcoholic drinks (especially beer) and beverages sweetened with high-fructose corn syrup.

Consumption of vegetables and lowfat or nonfat dairy products should be encouraged."

(Mei et al., 2017)

Chinese Multidisciplinary Expert Consensus on the Diagnosis and Treatment of Hyperuricemia and Related Diseases

"Encouraging a balanced diet, limiting total daily caloric intake, and controlling purine content in the diet are recommended. A low purine diet is encouraged [Table 1], but the intake of animal offal, seafood, meat, and other purine-rich foods should be strictly limited. Purine-rich vegetables (such as lettuce, spinach, mushrooms, and cauliflower), beans, and bean products are not significantly associated with HUA and gout attacks. Patients are encouraged to consume more fresh vegetables and appropriately consume beans and bean products (patients with renal insufficiency must consume such foods under the guidance of a specialist). (Choi et al., 2004)"

"Milk and dairy products (especially nonfat milk and low-calorie yogurt) can be consumed. The consumption of cola, orange juice, apple juice, and other fructose-containing beverages and sugar-containing soft drinks should be avoided"

". HUA patients can consume fruits with low fructose contents, such as cherries, strawberries, pineapple, watermelons, and peaches"

". Alcohol intake should be limited, and yellow rice wine, beer, and spirits should be avoided by HUA patients."

(Li et al., 2025)

Assessing cross-national inequalities and predictive trends in gout burden: a global perspective (1990–2021)

"A plant-based diet, rich in fruits, vegetables, whole grains, and legumes, provides a lower purine load while offering a variety of beneficial nutrients such as antioxidants, fiber, and essential vitamins (54, 55). For instance, the Mediterranean diet, known for its high intake of vegetables, fruits, legumes, nuts, and olive oil, is typically low in purines and has been shown to offer protective effects against gout and rheumatoid arthritis disease (Charneca et al., 2023)57). The consumption of plant-based foods not only reduces purine intake but also provides anti-inflammatory benefits that can help mitigate the severity of gout attacks (Thakuria et al., 2025)59)."

(Sahai et al., 2019)

Pharmacology of the Therapeutic Approaches of Gout

"Patients upon being diagnosed with hyperuricemia should be advised diet with less purine content (refined cereals, white bread, milk, peanut butter, fruits, nuts, tomato, green vegetables, etc.). Alcohol consumption should be kept at minimum; intake of whiskey and wine should be preferred rather than beer. Organ meats and beverages sweetened with high fructose corn syrup should be avoided. In addition, high intake of liquid diet should be advised to facilitate urine output of 2 L or more"

(Liang et al., 2024)

Analysis of the relationship of refractory gout between potential biomarkers and diet structure and lifestyle based on 1H-NMR

"A reasonable diet structures help prevent a gout attack"

".It is, therefore, necessary to control the total intake of both. Adjusting vegetablefruit dietary pattern can delay gout attack (Omar et al., 2021). We found that the gout group had lower vegetable intake than the healthy group, and the fruit intake was no statistically different relative to the healthy group. However, there were differences in fruit selection between the two groups. Gout patients tended to choose fruits with high sugar content, while high fructose intake was found to affect uric acid metabolism. The gout group had significantly higher meat intake than the healthy group, who were more inclined to eat high-purine food such as red meat, seafood, and animal offal. The intake of milk, eggs, and beans in the gout group was lower than that in the healthy group, while the intake of potatoes was not statistically different between the groups. However, the above-mentioned diets do not increase uric acid metabolism according to the literature (Rebholz et al., 2016)(2018)"

".advocate the reasonable distribution of food share on the basis of controlling the total calorie intake."

(Zhang et al., 2012)

Cherry Consumption and the Risk of Recurrent Gout Attacks

"Objective To study the relation between cherry intake and the risk of recurrent gout attacks among individuals with gout. Methods We conducted a case-crossover study to examine associations of a set of putative risk factors with recurrent gout attacks. Individuals with gout were prospectively recruited and followed online for one year. Participants were asked about the following information when experiencing a gout attack: the onset date of the gout attack, symptoms and signs, medications (including anti-gout medications), and potential risk factors (including daily intake of cherries and cherry extract) during the 2-day period prior to the gout attack. We assessed the same exposure information over 2-day control periods. We estimated the risk of recurrent gout attacks related to cherry intake using conditional logistic regression. Results Our study included 633 individuals with gout. Cherry intake over a 2-day period was associated with a 35% lower risk of gout attacks compared with no intake (multivariate odds ratio [OR] = 0.65, 95% CI: 0.50-0.85). Cherry extract intake showed a similar inverse association (multivariate OR=0.55, 95% CI: 0.30-0.98). The effect of cherry intake persisted across subgroups by sex, obesity status, purine intake, alcohol use, diuretic use, and use of anti-gout medications. When cherry intake was combined with allopurinol use, the risk of gout attacks was 75% lower than periods without either exposure (OR=0.25, 95% CI: 0.15-0.42). Conclusions These findings suggest that cherry intake is associated with a lower risk of gout attacks."

(Choi et al., 2009)

Vitamin C Intake and the Risk of Gout in Men – A Prospective Study

"Background Several metabolic studies and a recent double-blinded placebo-controlled randomized trial have shown that higher vitamin C intake significantly reduces serum uric acid levels. Yet, the relation with the risk of gout is unknown. Methods We prospectively examined over a 20-year period (1986–2006) the relation between vitamin C intake and the risk of incident gout in 46,994 male participants with no history of gout at baseline. We used a supplementary questionnaire to ascertain the American College of Rheumatology criteria for gout. Vitamin C intake was assessed every four years through validated questionnaires. Results During the 20 years of follow-up, we documented 1,317 confirmed incident cases of gout. Compared with men with vitamin C intake < 250mg/day, the multivariate relative risk (RR) of gout was 0.83 (95% confidence interval [CI], 0.71 to 0.97) for total vitamin C intake 500–999 mg/day, 0.66 (0.52 to 0.86) for 1,000–1,499 mg/day, and 0.55 (0.38 to 0.80) for ≥ 1500 mg/day (P for trend < 0.001). The multivariate RR per 500mg increase in total daily vitamin C intake was 0.83 (95% CI, 0.77 to 0.90). Compared with men who did not use supplemental vitamin C, the multivariate RR of gout was 0.66 (95% CI, 0.49 to 0.88) for supplemental vitamin C intake 1,000–1,499 mg/day and 0.55 (0.36 to 0.86) for ≥ 1500 mg/day (P for trend < 0.001). Conclusions Higher vitamin C intake is independently

associated with a lower risk of gout. Supplemental vitamin C intake may be beneficial in the prevention of gout."

(Chonin et al., 2021)

Research progress of hyperuricemia on kidney damage

"First, drink more water and drink less broth. For those with high serum uric acid and those with gout, drink more plain boiled water and less broth, fish broth, chicken broth, hotpot broth, etc"

"Secondly, eat more alkaline foods and less acidic foods. Patients with gout have purine metabolism disorders and abnormal uric acid, which can worsen the condition if they eat more acidic foods, and are not conducive to rehabilitation. However, eat more alkaline foods"

"Finally, eat more vegetables and less. Eat more vegetables, favor less purine intake, increase vitamin C, and increase cellulose."

Foods and beverages to limit or avoid

The main things to cut back are alcohol, sugar-sweetened or high-fructose drinks, and high-purine animal foods. The clearest food triggers are beer and other alcohol, organ meats, red meat, shellfish, and some seafood such as sardines and anchovies. (33 sources)

- **Alcohol: limit strongly, and avoid if it often triggers flares.** Alcohol is one of the most consistent diet risks in gout guidance and studies, and risk rises even with short-term intake before an attack. Beer is often singled out, but wine and liquor can also increase flare risk, so patients should not assume wine is "safe." ([Khanna, 2016](#)) ([Vedder et al., 2019](#)) ([Park et al., 2022](#)) ([Yip et al., 2023](#)) ([Hassan et al., 2023](#)) ([Lamb et al., 2023](#)) ([McCarty et al., 2025](#)) ([Neogi et al., 2014](#))
- **Beer and spirits deserve extra caution.** Several reviews and guidelines highlight beer in particular, and some also point to spirits as a common problem. Older advice sometimes preferred wine over beer, but stronger prospective evidence shows that all major alcohol types can raise attack risk. ([Sahai et al., 2019](#)) ([Mei et al., 2017](#)) ([Kim et al., 2017](#)) ([Hassan et al., 2023](#)) ([Neogi et al., 2014](#))
- **Avoid sugar-sweetened drinks and foods rich in fructose or high-fructose corn syrup.** This includes regular soda and other sweet drinks, and many guidelines also warn about fruit juice and sweetened soft drinks. These drinks are repeatedly linked with higher uric acid or higher gout risk. ([Sahai et al., 2019](#)) ([Singh et al., 2016](#)) ([Khanna, 2016](#)) ([M. et al., 2020](#)) ([Park et al., 2022](#)) ([Yip et al., 2023](#)) ([Timsans et al., 2024](#)) ([Sun et al., 2025](#)) ([Choi et al., 2010](#)) ([Ayoub-Charette et al., 2019](#))
- **Regular soda is a notable item to avoid.** Prospective cohort data found higher gout risk with increasing intake of sugar-sweetened soda, and some studies also found higher risk with orange juice or fruit juice, even though whole fruit does not show the same signal. ([Bitik et al., 2014](#)) ([Choi et al., 2010](#)) ([Kwiatkowska et al., 2020](#)) ([Ayoub-Charette et al., 2019](#))
- **Limit high-purine animal foods.** The most important foods here are organ meats, red meat, shellfish, and certain fish and seafood. Purine-rich animal foods are well linked with higher uric acid, higher gout risk, and recurrent flares. ([Khanna, 2016](#)) ([Bitik et al., 2014](#)) ([Filip et al., 2020](#)) ([Yip et al., 2023](#)) ([Hassan et al., 2023](#)) ([Yao et al., 2024](#)) ([Taju et al., 2024](#)) ([Saag et al., 2006](#)) ([Pande, 2006](#))
- **Especially avoid organ meats and meat extracts.** Liver, kidney, other offal, rich broths, and meat extracts are repeatedly listed among the highest-purine foods. ([Sahai et al., 2019](#)) ([Kaneko et al., 2014](#)) ([Chen et al., 2024](#)) ([Peixoto et al., 2001](#))
- **Some seafood is a common trigger.** Shellfish and fish such as sardines, anchovies, herring, and mackerel are often listed as foods to reduce because of their high purine content. ([Filip et al., 2020](#)) ([Hassan et al., 2023](#)) ([Chen et al., 2024](#)) ([Yao et al., 2024](#)) ([Peixoto et al., 2001](#))
- **Large short-term "purine loads" can trigger attacks.** In people who already have gout, higher purine intake over just the previous 2 days was linked with much higher odds of a flare, especially when the purines came from animal foods. ([Bitik et al., 2014](#)) ([Zhang et al., 2012](#))

- **Heavy meals and Western-style eating patterns are worth cutting back.** Guidance from EULAR includes avoiding heavy meals, and Western dietary patterns high in red/processed meat, sweets, refined grains, and sweetened beverages are linked with higher gout risk. ([Vedder et al., 2019](#)) ([Golenbiewski et al., 2019](#)) ([Chen et al., 2025](#)) ([Rai et al., 2017](#))
- **Do not over-restrict vegetables just because they contain purines.** This is an important exception: purine-rich vegetables generally do not show the same gout risk as animal purines, so most patients do not need to avoid vegetables like a traditional strict gout diet might suggest. ([Bitik et al., 2014](#)) ([Mei et al., 2017](#)) ([Susyani et al., 2017](#)) ([Zgaga et al., 2012](#))
- **Practical “avoid/limit” list for patients:**
- **Avoid or strongly limit:** beer, liquor, regular soda, sweetened soft drinks, high-fructose corn syrup products, organ meats, shellfish, sardines, anchovies, herring, mackerel, rich meat broths/extracts. ([Sahai et al., 2019](#)) ([Hassan et al., 2023](#)) ([Chen et al., 2024](#)) ([Kim et al., 2017](#))
- **Limit:** red meat, pork, lamb, beef, poultry, seafood in general, fruit juice, sweets, and other high-fructose foods or drinks. ([Singh et al., 2016](#)) ([Filip et al., 2020](#)) ([Yip et al., 2023](#)) ([Golenbiewski et al., 2019](#))

Evidence

(Khanna, 2016)

[Gout Transitions from Medieval Times into the 21st Century](#)

"Lifestyle measures and low-purine diet are cornerstones of preventing acute gout attacks and lowering urate levels. Purine breakdown from meat, seafood and alcohol are known to trigger gout flares. The guidelines outline clear dietary recommendations delineating dietary products as "avoid", "limit", or "encourage". Abstinence from alcohol consumption other than for cardio-protective purposes, weight loss, and avoidance of products rich in high fructose corn syrup are advocated based on epidemiologic data."

(Vedder et al., 2019)

[Dietary Interventions for Gout and Effect on Cardiovascular Risk Factors: A Systematic Review](#)

"The European League Against Rheumatism (EULAR) recommendations for the treatment of gout recommend lifestyle advice for all gout patients. Advice on diet includes the avoidance of alcohol, sugar-sweetened drinks, heavy meals, and excessive intake of meat and seafood, reflecting a purine-low diet (Richette et al., 2014)"

". Cardona et al. (2005) focused on subjects with gout and currently not on ULT and implemented a low-purine diet for two weeks [22]. Subjects were instructed to avoid certain foods, such as meat, poultry, fish, alcohol, and some legumes/vegetables."

(Park et al., 2022)

[Current state and prospects of gout treatment in Korea](#)

"Hence, it is recommended that patients with gout limit their alcohol intake regardless of disease activity (Fitzgerald et al., 2020). Regarding other dietary factors, a purine diet was reportedly associated with an increased risk of gout flares (Zhang et al., 2012), and a high-fructose diet was associated with a high risk of incident gout (Choi et al., 2010). Indeed, purine intake and a high-fructose diet should be limited in patients with gout (Fitzgerald et al., 2020)"

".it is better to focus on treatment using ULT and restrict mainly alcohol and high-fructose intake [17](Yokose et al., 2021). Weight loss approaches are also conditionally recommended for obese or overweight patients (Fitzgerald et al., 2020)."

(Yip et al., 2023)

[The Hong Kong Society of Rheumatology consensus recommendations for the management of gout](#)

"Patients should be advised to avoid alcohol, fructose, or sugar-sweetened beverages, and decrease dietary protein source from meat and seafood"

". A diet that restricts alcohol and purine-rich and fructose-rich foods, while maintaining the intake of low-fat dairy products, healthy protein sources, and vegetables, has been observed to lower the risk of incident gout [122]."

(Hassan et al., 2023)

[Gout: A Reviewing of The Recent Literature](#)

"To reduce the likelihood of recurrent flares, patients should limit their consumption of certain purine-rich foods (e.g., organ meats, shellfish) and avoid alcoholic drinks (especially beer) and beverages sweetened with high-fructose corn syrup.

Consumption of vegetables and lowfat or nonfat dairy products should be encouraged."

(Lamb et al., 2023)

[A content analysis of online videos containing dietary recommendations for gout and their alignment with evidence-based dietary guidelines](#)

"Dietary recommendations for gout in these guidelines concur and include restricting most purine-and fructose-rich foods and drinks, limiting alcohol consumption, particularly beer and spirits, remaining hydrated, eating sufficient dairy, and encouraging the consumption of vegetables and fruit (Hui et al., 2017)(Richette et al., 2014)12) ."

(McCarty et al., 2025)

[Gout therapy updated](#)

"Per the 2020 ACR/EULAR gout management recommendations, dietary limitation of alcohol, high-fructose corn syrup, and purine-rich foods in conjunction with weight reduction for obese or overweight patients with gout is highly recommended for chronic management of gout. (Fitzgerald et al., 2020)"

"Several diets, including low-fat, Mediterranean, low-carbohydrate and the Dietary Approaches to Stop Hypertension (DASH), have demonstrated reduced urate plasma levels"

". Meat, fish, and seafood have high purine content, which can be a risk factor for hyperuricemia and potential triggers of gout flares. (Abhishek et al., 2017) Additionally, food and beverages with high-fructose corn syrup can increase gout risk. Increased alcohol intake is also associated with hyperuricemia, particularly beer."

(Neogi et al., 2014)

[Alcohol quantity and type on risk of recurrent gout attacks: An internet-based case-crossover study](#)

"Objectives Although beer and liquor have been associated with risk of incident gout, wine has not. Yet anecdotally, wine is thought to trigger gout attacks. Further, how much alcohol intake is needed to increase the risk of gout attack is not known. We examined the quantity and type of alcohol consumed on risk of recurrent gout attacks. Methods We conducted a prospective internet-based case-crossover study in the United States among participants with gout and who had at least one attack during the one year of follow-up. We evaluated the association of alcohol intake over the prior 24 hours as well as the type of alcoholic beverage with risk of recurrent gout attack, adjusting for potential time-varying confounders. Results This study included 724 participants with gout (78% men, mean age 54 years). There was a significant dose-response relationship between amount of alcohol consumption and risk of recurrent gout attacks ($p < 0.001$ for trend). The risk of recurrent gout attack was 1.36 (95% CI: 1.00 to 1.88) and 1.51 (95% CI: 1.09 to 2.09) times higher for $>1-2$ and $>2-4$ alcoholic beverages, respectively, compared with no alcohol consumption in the prior 24 hours. Consuming wine, beer, or liquor, was each associated with an increased risk of gout attack. Conclusions Episodic alcohol consumption, regardless of type of alcoholic beverage, was associated with an increased risk of recurrent gout attacks, including potentially with moderate amounts. Persons with gout should limit alcohol intake of all types to reduce the risk of recurrent gout attacks."

(Sahai et al., 2019)

[Pharmacology of the Therapeutic Approaches of Gout](#)

"Patients upon being diagnosed with hyperuricemia should be advised diet with less purine content (refined cereals, white bread, milk, peanut butter, fruits, nuts, tomato, green vegetables, etc.). Alcohol consumption should be kept at minimum; intake of whiskey and wine should be preferred rather than beer. Organ meats and beverages sweetened with high fructose corn syrup should be avoided. In addition, high intake of liquid diet should be advised to facilitate urine output of 2 L or more"

(Mei et al., 2017)

[Chinese Multidisciplinary Expert Consensus on the Diagnosis and Treatment of Hyperuricemia and Related Diseases](#)

"Encouraging a balanced diet, limiting total daily caloric intake, and controlling purine content in the diet are recommended. A low purine diet is encouraged [Table 1], but the intake of animal offal, seafood, meat, and other purine-rich foods should be strictly limited. Purine-rich vegetables (such as lettuce, spinach, mushrooms, and cauliflower), beans, and bean products are not significantly associated with HUA and gout attacks. Patients are encouraged to consume more fresh vegetables and appropriately consume beans and bean products (patients with renal insufficiency must consume such foods under the guidance of a specialist). (Choi et al., 2004)"

"Milk and dairy products (especially nonfat milk and low-calorie yogurt) can be consumed. The consumption of cola, orange juice, apple juice, and other fructose-containing beverages and sugar-containing soft drinks should be avoided"

"HUA patients can consume fruits with low fructose contents, such as cherries, strawberries, pineapple, watermelons, and peaches"

"Alcohol intake should be limited, and yellow rice wine, beer, and spirits should be avoided by HUA patients."

(Kim et al., 2017)

Diagnosis and Treatment of Inflammatory Joint Disease

"Foods high in purine include beer, animal organ meats, seafood, crustaceans and others. People with gout should abstain from drinking alcohol entirely because it can block uric acid excretion and accelerate uric acid synthesis. For management of gout, the 2012 ACR guidelines suggests to avoid all purine-rich foods, high fructose corn syrup-containing drinks and foods and excessive alcohol consumption. Limiting consumption of beef, lamb, pork, seafood, juice and sugar and intake of low-fat dairy foods and vegetables are recommended (Khanna et al., 2012)."

(Singh et al., 2016)

A cross-sectional internet-based patient survey of the management strategies for gout

"Dietary modifications recommended for gout include increase in low-fat dairy and Vitamin C, and reduction of total protein, alcohol intake and high-fructose drinks (Choi, 2010)(Choi, 2005)."

(M. et al., 2020)

Comparative Study of Uric Acid Levels between Rural and Urban Populations

"Hyperuricaemic patients are recommended to reduce food causing hyperuricemia such as high purine rich diets, sweetened and alcoholic beverages"

"Following DASH diet (dietary approaches to stop hypertension) help in decreasing uric acid levels in serum. This diet helps in intake of fruits, vegetables, low fat products, whole grains, limiting sugars, sweetened beverages and saturated fats."

(Timsans et al., 2024)

Gout and Hyperuricemia: A Narrative Review of Their Comorbidities and Clinical Implications

"Patients with gout and hyperuricemia should be advised to limit purine-rich foods, such as red meats, seafood, and legumes, and to avoid sugar-sweetened drinks and foods rich in fructose. Alcohol avoidance should be encouraged as well."

(Sun et al., 2025)

2024 Update of Chinese Guidelines for Diagnosis and Treatment of Hyperuricemia and Gout Part I: Recommendations for General Patients

"Dietary restrictions on high-purine foods, high-fructose products, and alcohol are recommended; while dietary fiber, tart cherry products, citric acid products, and skim milk are encouraged"

"Certain dietary patterns, such as the Mediterranean diet and DASH diet (the dietary pattern in Dietary Approaches to Stop Hypertension), are beneficial for patients with gout by improving hyperuricemia and metabolic syndrome. These diets are more beneficial compared to the western diet, which is characterized by high purine, fat, and calorie content (Tang et al., 2017)(Lubawy et al., 2023)."

(Choi et al., 2010)

Fructose-Rich Beverages and the Risk of Gout in Women

"Context Fructose-rich beverages such as sugar-sweetened soda and orange juice can increase serum uric acid levels and thus, the risk of gout, but prospective data on the relation is limited. Objective To examine the relation between intake of fructose-rich beverages and fructose and the risk of incident gout among women. Design Prospective cohort study over 22 years (1984-2006) Setting The Nurses' Health Study Participants 78,906 women with no history of gout at baseline who provided information on intake of beverages and fructose through validated food-frequency questionnaires. Main outcome measures Incident cases of gout that met the American College of Rheumatology survey criteria for gout. Results During 22 years of follow-up, we documented 778 confirmed incident cases of gout. Increasing intake of sugar-sweetened soda was independently associated with increasing risk of gout. Compared with consumption of <1 serving/month, the multivariate relative risk of gout was 1.74 (95% confidence interval [CI], 1.19 to 2.25) for 1 serving/day and 2.39 (1.34 to 4.26) for ≥ 2 serving/day (P for trend<0.001). The corresponding RRs for orange juice were 1.41 (95% CI, 1.03 to 1.93) and 2.42 (95% CI, 1.27 to 4.63) (P for trend=0.02). The absolute risk differences corresponding to these RRs were 36 and 68 cases per 100,000 person-years for sugar-sweetened soda and 14 and 47 per 100,000 cases person-years for orange juice. Diet soft drinks were not associated with the risk of gout (P for trend=0.27). Compared with the lowest quintile of fructose intake, the multivariate relative risk of gout in the top quintile was 1.62 (95% CI, 1.20 to 2.19) (P for trend=0.004) (risk difference of cases 28 per 100,000 person-years). Conclusion These prospective data suggest that consumption of fructose-rich beverages increases the risk of incident gout among women, although their contribution to the risk of gout in the population is

likely modest given the low incidence rate among women."

(Ayoub-Charette et al., 2019)

Important food sources of fructose-containing sugars and incident gout: a systematic review and meta-analysis of prospective cohort studies

"Objective Sugar-sweetened beverages (SSBs) are associated with hyperuricaemia and gout. Whether other important food sources of fructose-containing sugars share this association is unclear. Design To assess the relation of important food sources of fructose-containing sugars with incident gout and hyperuricaemia, we conducted a systematic review and meta-analysis of prospective cohort studies. Methods We searched MEDLINE, Embase and the Cochrane Library (through 13 September 2017). We included prospective cohort studies that investigated the relationship between food sources of sugar and incident gout or hyperuricaemia. Two independent reviewers extracted relevant data and assessed the risk of bias. We pooled natural-log transformed risk ratios (RRs) using the generic inverse variance method with random effects model and expressed as RR with 95% confidence intervals (CIs). The overall certainty of the evidence was assessed using the Grading of Recommendations Assessment, Development and Evaluation system. Results We identified three studies (1 54 289 participants, 1761 cases of gout), comparing the highest with the lowest level of exposure for SSBs, fruit juices and fruits. No reports were found reporting incident hyperuricaemia. Fruit juice and SSB intake showed an adverse association (fruit juice: RR=1.77, 95% CI 1.20 to 2.61; SSB: RR=2.08, 95% CI 1.40 to 3.08), when comparing the highest to lowest intake of the most adjusted models. There was no significant association between fruit intake and gout (RR 0.85, 95% CI 0.63 to 1.14). The strongest evidence was for the adverse association with SSB intake (moderate certainty), and the weakest evidence was for the adverse association with fruit juice intake (very low certainty) and lack of association with fruit intake (very low certainty). Conclusion There is an adverse association of SSB and fruit juice intake with incident gout, which does not appear to extend to fruit intake. Further research is needed to improve our estimates. Trial registration number NCT02702375; Results."

(Bitik et al., 2014)

An old disease with new insights: Update on diagnosis and treatment of gout.

"The association between a chronic purine-rich diet, mainly of animal origin, and hyperuricemia or incident gout is well established (Choi et al., 2005)(Villegas et al., 2012)(Choi et al., 2004)(Kaneko et al., 2014). It is also known that acute purine intake (over 2 days) increases the risk of recurrent gout attacks in patients with gout (Zhang et al., 2012). Fructose-rich beverages, such as sugar-sweetened soda, are reported to increase the risk of incident gout (Choi et al., 2010). Alcohol (including wine, beer, and liquor), sugar-sweetened soft drinks and, fructose consumption were found to be associated with an increased risk of gout (Choi et al., 2004)(Neogi et al., 2014)(Choi et al., 2008). Some foods and beverages are reported to have protective effects against gout. Dairy products, cherry consumption, or vitamin C intake are reported to decrease serum uric acid and the frequency of gout flares (Choi et al., 2005)(Zgaga et al., 2012)(Dalbeth et al., 2011)(Dalbeth et al., 2012)(Choi et al., 2009)(Zhang et al., 2012). The intake of purine-rich vegetables was not associated to plasma uric acid (Zgaga et al., 2012). A large prospective observational trial found that the risk of incident gout decreased with increasing coffee intake (>4-5 cups per day) (Choi et al., 2007)."

(Kwiatkowska et al., 2020)

The place of omega-3 and omega-6 acids in supplementary treatment of inflammatory joint diseases

"Treatment for gout usually includes pharmacological treatment and implementation of a diet aiming to reduce the purine content in consumed meals. Recent studies have shown that, similarly to other rheumatic diseases, the Mediterranean diet, namely a diet rich in polyunsaturated fatty acids, influences the level of uric acid and decreases the frequency of gout attacks (Stamostergiou et al., 2018)"

".The recently observed increasing number of patients suffering from gout results from the growing number of obese people consuming excessive amounts of meat, as well as from rising consumption of fructose (Ayoub-Charette et al., 2019)"

".Nonetheless, it is worth remembering the recommendation to limit the overall purine intake (Zhang et al., 2019)."

(Filip et al., 2020)

Gout – management of a chronic disease: a systematic review

"Patients with gout should be encouraged to have a well-balanced diet, rich in vegetables and fiber, low in fat and sugar and also to exercise.

The major risk factor for gout in men is weight gain so that weight loss is necessary, the same as stopping the use of high-fructose corn syrup, rich purine animal protein (organ meats, lamb, pork, beef, shellfish) and alcohol (Hainer et al., 2014). The diet is based on skim milk and/ or lowfat yogurt, vegetable sources of protein, including beans and soy, but also cherries (Hui et al., 2017)."

(Yao et al., 2024)

Advances in Gouty Arthritis Management: Integration of Established Therapies, Emerging Treatments, and Lifestyle Interventions

"A diet low in purine can result in a significant reduction in serum urate levels. Purinerich foods, such as red meat, organ meats (e.g.,

liver and kidney), shellfish, and certain types of seafood (e.g., sardines, anchovies, and mackerel), are metabolized into uric acid. Reducing the consumption of these foods helps lower uric acid production in the body, thereby reducing the risk of gout flares. Accordingly, patients are advised to consume lean meats, poultry, and plant-based protein sources such as legumes, which have lower purine content (Yokose et al., 2021)."

(Taju et al., 2024)

Role of Diet and Lifestyle in Prevention of Recurrent Gout Attacks

"High-purine foods, including red and organ meats and certain seafood, are significant contributors to elevated serum uric acid levels, while low-purine diets, dairy products, and plant-based proteins have protective effects. Fructose, commonly found in sugar-sweetened beverages, exacerbates hyperuricemia, whereas proper hydration and antioxidant-rich foods, such as cherries and citrus fruits, support urate clearance and reduce inflammation."

(Saag et al., 2006)

Epidemiology, risk factors, and lifestyle modifications for gout

"Thus, reducing intake of purine-rich foods, particularly those of animal origin such as red meat, is recommended in the prevention and management of hyperuricemia and gout"

"Thus, it is important for patients with gout to limit red meat consumption, as is indicated by the new Healthy Eating Pyramid recommendations for the general public (Fig. 2) (Willett et al., 2003)"

"For cardiovascular benefit, instead of consuming fish, patients with gout could consider taking plant-derived ω 3 fatty acids or eicosapentanoic acid and docosahexanoic acid supplements"

"The recent recommendations on dairy consumption for the general public may be applicable in most patients with gout or hyperuricemia, and may offer additional benefit to those individuals with hypertension, diabetes, and cardiovascular disorders (Choi et al., 2004)[56]."

(Pande, 2006)

An update on gout

"Patients with gout should avoid excess of alcohol (beer and spirits, particularly), red-meat and sea-food. Vegetarian food and dairy products reduce the risk of gouty arthritis."

(Kaneko et al., 2014)

Total purine and purine base content of common foodstuffs for facilitating nutritional therapy for gout and hyperuricemia.

"Purines are natural substances found in all of the body's cells and in virtually all foods. In humans, purines are metabolized to uric acid, which serves as an antioxidant and helps to prevent damage caused by active oxygen species. A continuous supply of uric acid is important for protecting human blood vessels. However, frequent and high intake of purine-rich foods reportedly enhances serum uric acid levels, which results in gout and could be a risk factor for cardiovascular disease, kidney disease, and metabolic syndrome. In Japan, the daily intake of dietary purines is recommended to be less than 400 mg to prevent gout and hyperuricemia. We have established an HPLC method for purine analysis and determined purines in a total of 270 foodstuffs. A relatively small number of foods contained concentrated amounts of purines. For the most part, purine-rich foods are also energy-rich foods, and include animal meats, fish meats, organs such as the liver and fish milt, and yeast. When the ratio of the four purine bases (adenine, guanine, hypoxanthine, and xanthine) was compared, two groups of foods were identified: one that contained mainly adenine and guanine and one that contained mainly hypoxanthine. For patients with gout and hyperuricemia, the amount of total purines and the types of purines consumed, particularly hypoxanthine, are important considerations. In this context, the data from our analysis provide a purine content reference, and thereby clinicians and patients could utilize that reference in nutritional therapy for gout and hyperuricemia."

(Chen et al., 2024)

Effect of low-purine diet on the serum uric acid of gout patients in different clinical subtypes: a prospective cohort study

"The recommended diet is low in fat (up to 30% of total caloric value), high in carbohydrates, low in protein (0.8 g/kg/day), and limited in the consumption of foods high in purines (100-1000 mg/100 g). These foods include viscera, consommé, meat extract, fish (like herring, mackerel, and sardines), mussels, anchovies, alcoholic beverages, yeast, foods containing eggs and yeast, and partridge. It is also advisable to limit consumption of foods with moderate purine content (9 to 100 mg per 100 g), including meats, seafood, and certain vegetables. Additionally, it is recommended to avoid foods high in oxalates, such as dark green vegetables, cauliflower, beet/chard, beetroot, eggplant, okra, sweet potatoes, chestnuts, coconuts, wheat germ, tomatoes, asparagus, mushrooms, strawberries, whole wheat cereals, quince marmalade, and chocolate. The recommended daily liquid intake is 2.5 to 3.5 L (Peixoto et al., 2001)."

(Peixoto et al., 2001)

Diet and medication in the treatment of hyperuricemia in hypertensive patients.

"The recommended diet was moderate in protein (0.8g/ kg/day), high in carbohydrates, low in fat (up to 30% of the total caloric value), and it restricted the consumption of food with a high purine content (100 to 1000mg of purine by 100g of food) such as consommé, meat extract, viscera, fish (such as herring, mackerel, and sardines), mussels, anchovies, alcoholic beverages, yeast, food in which yeast and eggs were used, and partridge 8"

".If the hyperuricemic patient already followed a diet with the previously mentioned characteristics, a decrease in the consumption of foods with moderate purine content (9 to 100mg of purine per 100g of food) such as meats of all types, seafood, and vegetables was recommended, as well as a decrease in the consumption of food high in oxalates, such as dark green vegetables, cauliflower, beet/chard, beetroot, eggplant, okra, sweet potatoes, chestnuts, coconuts, wheat germ, tomatoes, asparagus, mushrooms, strawberries, whole wheat cereals, quince marmalade, and chocolate 8"

".The recommended ingestion of liquids was 2.5 to 3.5 liters/day 8."

(Zhang et al., 2012)

Purine-rich foods intake and recurrent gout attacks

"Objective To examine and quantify the relation between purine intake and the risk of recurrent gout attacks among gout patients. Methods The authors conducted a case-crossover study to examine associations of a set of putative risk factors with recurrent gout attacks. Individuals with gout were prospectively recruited and followed online for 1 year. Participants were asked about the following information when experiencing a gout attack: the onset date of the gout attack, clinical symptoms and signs, medications (including antigout medications), and presence of potential risk factors (including daily intake of various purine-containing food items) during the 2-day period prior to the gout attack. The same exposure information was also assessed over 2-day control periods. Results This study included 633 participants with gout. Compared with the lowest quintile of total purine intake over a 2-day period, OR of recurrent gout attacks were 1.17, 1.38, 2.21 and 4.76, respectively, with each increasing quintile (p for trend <0.001). The corresponding OR were 1.42, 1.34, 1.77 and 2.41 for increasing quintiles of purine intake from animal sources (p for trend <0.001), and 1.12, 0.99, 1.32 and 1.39 from plant sources ($p=0.04$), respectively. The effect of purine intake persisted across subgroups by sex, use of alcohol, diuretics, allopurinol, NSAIDs and colchicine. Conclusions The study findings suggest that acute purine intake increases the risk of recurrent gout attacks by almost fivefold among gout patients. Avoiding or reducing amount of purine-rich foods intake, especially of animal origin, may help reduce the risk of gout attacks."

(Golenbiewski et al., 2019)

Moving the Needle: Improving the Care of the Gout Patient

"Many foods are associated with increased serum uric acid levels including red and processed meats, shellfish, alcoholic beverages, sugar-containing beverages and foods (including high-fructose corn syrup), and refined grains (Major et al., 2018)(Abhishek et al., 2018)(Rai et al., 2017). These should be consumed in moderation or avoided altogether, in some cases"

".Certain diets, including the Dietary Approaches to Stop Hypertension (DASH diet) and the Mediterranean diet have been shown to reduce serum urate levels and the risk of gout (Major et al., 2018), and are an appropriate preventative approach for those with hyperuricemia at increased risk of developing gout (Rai et al., 2017)."

(Chen et al., 2025)

Advances in Understanding the Mechanisms of Treatment for Gouty Arthritis: A Comprehensive Review

"Traditional dietary recommendations for gout have focused on limiting intake of purine-rich foods -such as organ meats, certain seafood, and beer -to reduce serum urate levels (Nielsen et al., 2018). While it remains important for individuals with gout to recognize and avoid foods that may specifically trigger their flares, emerging evidence indicates that the overall dietary pattern may be more beneficial than strict purine restriction alone"

".Recent large-scale studies and expert guidelines now favor the adoption of cardiometabolically healthy dietary patterns such as the Dietary Approaches to Stop Hypertension (DASH) diet and the Alternative Healthy Eating Index (AHEI), which are associated with lower serum urate levels, reduced risk of gout, and improvement in comorbidities like hypertension and insulin resistance (Rai et al., 2017)(Yokose et al., 2022). The Mediterranean diet, which emphasizes fruit, vegetables, whole grains, healthy fats, and moderate dairy intake, has also been suggested as beneficial in observational studies (Dessein et al., 2000)"

".Alcohol consumption: Avoidance of alcohol, particularly beer and spirits, is a key aspect of dietary management in gout."

(Rai et al., 2017)

The Dietary Approaches to Stop Hypertension (DASH) diet, Western diet, and risk of gout in men: prospective cohort study

"Objective To prospectively examine the relation between the Dietary Approaches to Stop Hypertension (DASH) and Western diets and risk of gout (ie, the clinical endpoint of hyperuricemia) in men. Design Prospective cohort study. Setting The Health Professionals Follow-up Study. Participants 44 444 men with no history of gout at baseline. Using validated food frequency

questionnaires, each participant was assigned a DASH dietary pattern score (based on high intake of fruits, vegetables, nuts and legumes, low fat dairy products, and whole grains, and low intake of sodium, sweetened beverages, and red and processed meats) and a Western dietary pattern score (based on high intake of red and processed meats, French fries, refined grains, sweets, and desserts). Main outcome measure Risk of incident gout meeting the preliminary American College of Rheumatology survey criteria for gout, adjusting for potential confounders, including age, body mass index, hypertension, diuretic use, and alcohol intake. Results During 26 years of follow-up, 1731 confirmed cases of incident gout were documented. A higher DASH dietary pattern score was associated with a lower risk for gout (adjusted relative risk for extreme fifths 0.68, 95% confidence interval 0.57 to 0.80, P value for trend <0.001). In contrast, a higher Western dietary pattern score was associated with an increased risk for gout (1.42, 1.16 to 1.74, P=0.005). Conclusion The DASH diet is associated with a lower risk of gout, suggesting that its effect of lowering uric acid levels in individuals with hyperuricemia translates to a lower risk of gout. Conversely, the Western diet is associated with a higher risk of gout. The DASH diet may provide an attractive preventive dietary approach for men at risk of gout."

(Susyani et al., 2017)

Nutrition Counseling among Patients with Gout

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". Consuming of meat and seafood in high amounts shown to increasing of uric acid levels (Choi et al., 2005), while the consuming protein or vegetables rich in purine in moderation was not shown to increase the risk of gout patients (Choi et al., 2004)"

". The main principle of nutrition therapy in patients with gout and hyperuricemia is including restrictions on the amount of purines in the diet and the proper regulation of body weight [6]"

". patients change their behaviors and avoiding the consuming foods that contain high in purines that can increase the levels of uric acid"

". Gout patients need to maintain diet and adhere to diets recommended by nutritionists. Nutritionists need to improve nutritional counseling to Gout patients, because nutrition counseling with Low-purine diet in patients with Gout can change the eating habits of patients who eventually decreased the levels of uric acid."

(Zgaga et al., 2012)

The Association of Dietary Intake of Purine-Rich Vegetables, Sugar-Sweetened Beverages and Dairy with Plasma Urate, in a Cross-Sectional Study

"Introduction Hyperuricemia is a strong risk factor for gout. The incidence of gout and hyperuricemia has increased recently, which is thought to be, in part, due to changes in diet and lifestyle. Objective of this study was to investigate the association between plasma urate concentration and: a) food items: dairy, sugar-sweetened beverages (SSB) and purine-rich vegetables; b) related nutrients: lactose, calcium and fructose. Methods A total of 2,076 healthy participants (44% female) from a population-based case-control study in Scotland (1999–2006) were included in this study. Dietary data was collected using a semi-quantitative food frequency questionnaire (FFQ). Nutrient intake was calculated using FFQ and composition of foods information. Urate concentration was measured in plasma. Results Mean urate concentration was 283.8±72.1 mmol/dL (females: 260.1±68.9 mmol/dL and males: 302.3±69.2 mmol/dL). Using multivariate regression analysis we found that dairy, calcium and lactose intakes were inversely associated with urate (p=0.008, p=0.003, p=0.0007, respectively). Overall SSB consumption was positively associated with urate (p=0.008), however, energy-adjusted fructose intake was not associated with urate (p=0.66). The intake of purine-rich vegetables was not associated to plasma urate (p=0.38). Conclusions Our results suggest that limiting purine-rich vegetables intake for lowering plasma urate may be ineffectual, despite current recommendations. Although a positive association between plasma urate and SSB consumption was found, there was no association with fructose intake, suggesting that fructose is not the causal agent underlying the SSB-urate association. The abundant evidence supporting the inverse association between plasma urate concentration and dairy consumption should be reflected in dietary guidelines for hyperuricemic individuals and gout patients. Further research is needed to establish which nutrients and food products influence plasma urate concentration, to inform the development of evidence-based dietary guidelines."

Lifestyle and implementation considerations

Diet changes for gout work best when they are practical and long term, not extreme or short lived. The most useful habits are steady weight loss if overweight, good hydration, limiting alcohol and fructose, and using diet together with urate-lowering treatment when needed. (13 sources)

Lifestyle advice is recommended for essentially all patients with gout, but the key is to make it sustainable in day-to-day life rather than trying to follow an overly strict "gout diet" for a short time. Reviews and guidelines consistently support focusing on a few high-yield changes: limit alcohol, avoid sugar-sweetened and other high-fructose drinks, reduce high-purine animal foods, avoid heavy meals, and work toward a healthier body weight if overweight. (Vedder et al., 2019) (Park et al., 2022) (Fitzgerald et al., 2020) (Khanna, 2016) (Saliba et

[al., 2024\)](#)

Weight management is an important part of implementation. For patients with overweight or obesity, gradual weight loss is commonly recommended because it can help both gout and related conditions such as hypertension, insulin resistance, and metabolic syndrome. At the same time, the overall message from recent reviews is not to rely on diet alone when gout is established; for many patients it is better to combine healthy eating with urate-lowering therapy instead of expecting strict food avoidance by itself to fully control disease. [\(Park et al., 2022\)](#) [\(Yokose et al., 2021\)](#) [\(Tomaszuk et al., 2023\)](#) [\(Saliba et al., 2024\)](#)

Hydration is also a practical part of care. Several sources advise adequate fluid intake, and older diet protocols often recommended roughly 2.5 to 3.5 liters per day, though the right amount should still fit the person's heart, kidney, and overall health status. [\(Saliba et al., 2024\)](#) [\(Chen et al., 2024\)](#) [\(Peixoto et al., 2001\)](#)

In real-world counseling, it can help to frame the diet as "mostly healthy eating, with a short list of things to really watch." That means building meals around DASH- or Mediterranean-style patterns when possible, then paying special attention to common triggers such as alcohol binges, sugary drinks, and large meat or seafood loads over a day or two, which can raise flare risk quickly. [\(Tomaszuk et al., 2023\)](#) [\(Yokose et al., 2021\)](#) [\(Park et al., 2022\)](#) [\(Zhang et al., 2012\)](#)

Finally, follow-through matters. Nutrition counseling and repeated reinforcement can improve adherence and help patients change eating habits over time, which may lower uric acid and reduce flares. [\(Susyani et al., 2017\)](#) [\(Zhelyabina et al., 2024\)](#) [\(Timsans et al., 2024\)](#)

Evidence

[\(Vedder et al., 2019\)](#)

[Dietary Interventions for Gout and Effect on Cardiovascular Risk Factors: A Systematic Review](#)

"The European League Against Rheumatism (EULAR) recommendations for the treatment of gout recommend lifestyle advice for all gout patients. Advice on diet includes the avoidance of alcohol, sugar-sweetened drinks, heavy meals, and excessive intake of meat and seafood, reflecting a purine-low diet (Richette et al., 2014)"

". Cardona et al. (2005) focused on subjects with gout and currently not on ULT and implemented a low-purine diet for two weeks [22]. Subjects were instructed to avoid certain foods, such as meat, poultry, fish, alcohol, and some legumes/vegetables."

[\(Park et al., 2022\)](#)

[Current state and prospects of gout treatment in Korea](#)

"Hence, it is recommended that patients with gout limit their alcohol intake regardless of disease activity (Fitzgerald et al., 2020). Regarding other dietary factors, a purine diet was reportedly associated with an increased risk of gout flares (Zhang et al., 2012), and a high-fructose diet was associated with a high risk of incident gout (Choi et al., 2010). Indeed, purine intake and a high-fructose diet should be limited in patients with gout (Fitzgerald et al., 2020)"

".it is better to focus on treatment using ULT and restrict mainly alcohol and high-fructose intake [17](Yokose et al., 2021). Weight loss approaches are also conditionally recommended for obese or overweight patients (Fitzgerald et al., 2020)."

[\(Fitzgerald et al., 2020\)](#)

[2020 American College of Rheumatology Guideline for the Management of Gout](#)

"To provide guidance for the management of gout, including indications for and optimal use of urate-lowering therapy (ULT), treatment of gout flares, and lifestyle and other medication recommendations."

[\(Khanna, 2016\)](#)

[Gout Transitions from Medieval Times into the 21st Century](#)

"Lifestyle measures and low-purine diet are cornerstones of preventing acute gout attacks and lowering urate levels. Purine breakdown from meat, seafood and alcohol are known to trigger gout flares. The guidelines outline clear dietary recommendations delineating dietary products as "avoid", "limit", or "encourage". Abstinence from alcohol consumption other than for cardio-protective purposes, weight loss, and avoidance of products rich in high fructose corn syrup are advocated based on epidemiologic data."

[\(Saliba et al., 2024\)](#)

Treatment of Gout in Patients with CrCl $\leq$ 30 mL/min and/or on Hemodialysis: A Review

"Advice may include limiting alcohol, high-purine foods, sugar-sweetened drinks, encouraging weight loss if overweight, low-fat dairy, cherries, and adequate fluids"

"Weight loss and avoiding certain dietary triggers appear beneficial for gout"

(Yokose et al., 2021)

The Role of Diet in Hyperuricemia and Gout

"Purpose of review Although gout's cardinal feature is inflammatory arthritis, it is closely associated with insulin resistance and considered a manifestation of the metabolic syndrome. As such, both gout and hyperuricemia are often associated with major cardiometabolic and renal comorbidities that drive the persistently elevated premature mortality rates among gout patients. To that end, conventional low-purine (i.e., low-protein) dietary advice given to many patients with gout warrant reconsideration. Recent findings Recent research suggests that several healthy diets, such as the Mediterranean or Dietary Approaches to Stop Hypertension (DASH) diets, in combination with weight loss for those who are overweight or obese, can drastically improve cardiometabolic risk factors and outcomes. By treating gout as a part of the metabolic syndrome and shifting our dietary recommendations to these healthy dietary patterns, the beneficial effects on gout endpoints should naturally follow for the majority of typical gout cases, mediated through changes in insulin resistance. Summary Dietary recommendations for the management of hyperuricemia and gout should be approached holistically, taking into consideration its associated cardiometabolic comorbidities. Several healthy dietary patterns, many with similar themes, can be tailored to suit comorbidity profiles and personal preferences."

(Tomaszuk et al., 2023)

Nutritional factors in the prevention and treatment of gout

"Studies to date suggest that avoidance of certain foods and beverages can decrease the frequency of gout flares. Weight loss may be beneficial for prevention as well as treatment of gout and its comorbidities. The impact of various types of diet on the course of gout has been given particular attention and recent research suggests that vegetarian, mediterranean and dash diets may be beneficial for gouty patients."

(Chen et al., 2024)

Effect of low-purine diet on the serum uric acid of gout patients in different clinical subtypes: a prospective cohort study

"The recommended diet is low in fat (up to 30% of total caloric value), high in carbohydrates, low in protein (0.8 g/kg/day), and limited in the consumption of foods high in purines (100-1000 mg/100 g). These foods include viscera, consommé, meat extract, fish (like herring, mackerel, and sardines), mussels, anchovies, alcoholic beverages, yeast, foods containing eggs and yeast, and partridge. It is also advisable to limit consumption of foods with moderate purine content (9 to 100 mg per 100 g), including meats, seafood, and certain vegetables. Additionally, it is recommended to avoid foods high in oxalates, such as dark green vegetables, cauliflower, beet/chard, beetroot, eggplant, okra, sweet potatoes, chestnuts, coconuts, wheat germ, tomatoes, asparagus, mushrooms, strawberries, whole wheat cereals, quince marmalade, and chocolate. The recommended daily liquid intake is 2.5 to 3.5 L (Peixoto et al., 2001)."

(Peixoto et al., 2001)

Diet and medication in the treatment of hyperuricemia in hypertensive patients.

"The recommended diet was moderate in protein (0.8g/ kg/day), high in carbohydrates, low in fat (up to 30% of the total caloric value), and it restricted the consumption of food with a high purine content (100 to 1000mg of purine by 100g of food) such as consommé, meat extract, viscera, fish (such as herring, mackerel, and sardines), mussels, anchovies, alcoholic beverages, yeast, food in which yeast and eggs were used, and partridge 8"

"If the hyperuricemic patient already followed a diet with the previously mentioned characteristics, a decrease in the consumption of foods with moderate purine content (9 to 100mg of purine per 100g of food) such as meats of all types, seafood, and vegetables was recommended, as well as a decrease in the consumption of food high in oxalates, such as dark green vegetables, cauliflower, beet/chard, beetroot, eggplant, okra, sweet potatoes, chestnuts, coconuts, wheat germ, tomatoes, asparagus, mushrooms, strawberries, whole wheat cereals, quince marmalade, and chocolate 8"

"The recommended ingestion of liquids was 2.5 to 3.5 liters/day 8."

(Zhang et al., 2012)

Purine-rich foods intake and recurrent gout attacks

"Objective To examine and quantify the relation between purine intake and the risk of recurrent gout attacks among gout patients. Methods The authors conducted a case-crossover study to examine associations of a set of putative risk factors with recurrent gout attacks. Individuals with gout were prospectively recruited and followed online for 1 year. Participants were asked about the following information when experiencing a gout attack: the onset date of the gout attack, clinical symptoms and signs, medications

(including antigout medications), and presence of potential risk factors (including daily intake of various purine-containing food items) during the 2-day period prior to the gout attack. The same exposure information was also assessed over 2-day control periods. **Results** This study included 633 participants with gout. Compared with the lowest quintile of total purine intake over a 2-day period, OR of recurrent gout attacks were 1.17, 1.38, 2.21 and 4.76, respectively, with each increasing quintile (p for trend <0.001). The corresponding OR were 1.42, 1.34, 1.77 and 2.41 for increasing quintiles of purine intake from animal sources (p for trend <0.001), and 1.12, 0.99, 1.32 and 1.39 from plant sources ($p=0.04$), respectively. The effect of purine intake persisted across subgroups by sex, use of alcohol, diuretics, allopurinol, NSAIDs and colchicine. **Conclusions** The study findings suggest that acute purine intake increases the risk of recurrent gout attacks by almost fivefold among gout patients. Avoiding or reducing amount of purine-rich foods intake, especially of animal origin, may help reduce the risk of gout attacks."

(Susyani et al., 2017)

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(Zhelyabina et al., 2024)

Diet for gout and hyperuricaemia: some important questions

"Diet has traditionally been viewed as playing a leading role in both the pathogenesis and treatment of gout. And although this thesis is controversial today, adherence to certain dietary rules for patients with gout and hyperuricemia (HU) is an integral part of therapy. The review examines the modern theoretical basis of dietary therapy for gout and HU, in particular the mechanisms of increasing serum uric acid levels and the risk of developing arthritis when certain foods are consumed and, conversely, reducing uric acid levels and the risk of gout when a diet containing a range of vitamins and foods in the supplements is followed."

(Timsans et al., 2024)

Gout and Hyperuricemia: A Narrative Review of Their Comorbidities and Clinical Implications

"Patients with gout and hyperuricemia should be advised to limit purine-rich foods, such as red meats, seafood, and legumes, and to avoid sugar-sweetened drinks and foods rich in fructose. Alcohol avoidance should be encouraged as well."